



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : روجيه حسن طه حسن
المحافظة : القليوبية الحي / المدينة : القناطر الخيرية العنوان : 0
رقم البطاقة : 28403071401947 رقم التليفون : 01010856281 تاريخ تسجيل الإستمارة : 24-09-2026
العمر (تاريخ الميلاد) : 07-03-1984 المهنة : ربه منزل

الإقرار

أقر أنا الموقع أدناه أنني لا اتمتع بأي نظام علاج مثل:- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات أو أي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإحصائي الاجتماعي
التوقيع: (-----) المقر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام مزيد من الأبحاث و الاشعات [7954] قسم العلاج : الاورام
التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني
الاسم: د/ندى حسن
الاسم: د/نرمين رمضان
الوظيفة: -----
الوظيفة: -----
الوظيفة: -----
التوقيع: -----
التوقيع: -----
التوقيع: -----
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير المستشفى.
تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

د. / ----- د. / ----- د. / ----- د. / -----

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى :
جهة العلاج :
ملاحظات :
قسم :
نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

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Name: روجيه حسن طه حسن National ID: 28403071401947	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Further investigations and imaging

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين





Verified by :
 Dr. Heba Hasan

Diagnosis

- * KI 67: 15 % nuclear reaction of tumor cells
- * HER2: Faint incomplete membrane reaction in >10 % of tumor cells (score 1+)
- * PR: Strong nuclear reaction in 80 % tumor cells (Allred score 8/8)
- * ER: Strong nuclear reaction in 95 % tumor cells (Allred score 8/8)
- * P63 & Calponin, SMA: Positive in DCIS , Negative in invasive component
- * IPT (Positive control showed appropriate reactivity) :
- * Perineural invasion: No
- * Angioinvasion: Couldn't be detected in the received biopsy
- * DCIS: Focal
- * Grade (differentiation + nuclear pleomorphism + mitosis): (3+2+2) Grade II
- * Histological type: Invasive duct carcinoma
- * Microscopic examination

Gross examination
 Multiple grayish white tissue cores up to 2 cm, totally submitted.
 IHC technique: Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for ER, PR, HER2, ER clone (1E2), PR clone (SP1), HER-2 clone (4B5), & Ki67 clone (30-9).

Nature of specimen
 Tru cut biopsy, collected at 25/3/2025, 10:30 am

Clinical Data
 Right breast mass

Pathology Report

Referred by :
 Patient Name : روجية حسن طه حسن : 28403071401947
 Patient No. : 1161/25
 Sex : Female
 Age : 41 Y
 Department : Out Patient
 Registered by : marwa mohamed youss
 Request No. : 13002724
 Request Date : 26/03/2025
 Verification Date : 22/04/2025
 Financial Category : مريض - مريض : 26/03/2025
 اورام دار السلام

Patient Name : الأستاذه / روجيه حسن طه حسن

Patient No. : 28403071401947

Pain No. : 1161/25

Sex : Female

Age : 41 Y

Referring Doc :

Request No. : 13002724

Request Date : 26/03/2025

Verification Date : 22/04/2025

Financial Category : مبادره - اورام دار السلام

Department : Out Patient

Registered by : .marwa mohamed youss

Pathology Report

Right breast mass, Tru cut biopsy:

Invasive duct carcinoma, Grade II

IPI:

*ER: Positive (score 8/8).

*PR: Positive (score 8/8).

*HER2: (score 1+).

*Ki67: Positive in 15% of tumor cells.

Signature

Dr. Walaa Ghoneim

Dr.Heba Hassan

Verified by :

Dr.Heba Hasan

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Page 2 of 2

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Patient Name: Rawheya Hassan Taha Hassan	Date: 15/05/2025
Exam: C.T. CHEST WITH CONTRAST	Age: 41 Year(s)
Medical ID: 1245/2025	Pat. ID: 98606

POST-CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN AND PELVIS:

- Reported case of breast cancer.

I Chest:

- Few bilateral pulmonary nodules the largest is left fissural based measuring 4 mm.
- Right lower lobe medial basal atelectatic changes.
- Few prominent subcentimetric mediastinal lymph nodes.
- No pleural or pericardial collection, pleural thickening or masses.
- No cardiac gross enlargement.

II Abdomen & pelvis:

- Average sized liver showing homogenous parenchymal attenuation. No sizable focal lesions.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal CT appearance of the spleen, pancreas, kidneys, suprarenal glands, aorta and IVC.
- Averagely filled urinary bladder showing no sizable diverticular outpouching.
- No pathological abdominal or pelvic lymph node enlargement.
- No ascites.
- Right iliac wing sclerotic focus mostly bone island.

Opinion:

Reported case of breast cancer revealed:

- Bilateral pulmonary nodules for follow up.
- No sizable hepatic lesions.

Dr. Mohamed Mohsen, M.D.



Patient Name: Rawheya Hassan Taha Hassan	Date : 24/03/2025
Exam : ultrasound BREAST (BOTH SIDE)	Age : 41 Year(s)
Medical ID: 28403071401947	Pat. ID: 98606

**DIGITAL BILATERAL SOFT TISSUE MAMMOGRAPHY AND
COMPLEMENTARY ULTRASOUND**

Indication:

Right breast palpable mass

Technique

- Bilateral MLO and CC views
- Bilateral 3D MLO DBT (Digital Breast tomosynthesis)
- Bilateral breast ultrasound

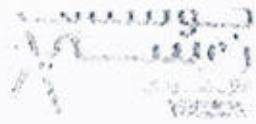
Report:

- ACR B
- Right Breast:
 - Right breast global asymmetry with overlying suspicious pleomorphic micro-calcifications reaching the nipple, US showed area of altered parenchyma.
 - No pathologically enlarged axillary LNs.
 - No pathological enlarged infra or supraclavicular lymph nodes.
- Left Breast:
 - No suspicious micro calcifications or architectural distortion detected.
 - Complementary ultrasound no cystic or solid mass lesions.
 - Normal skin thickness, and nipple-areola complex with average caliber of the retro-areolar central ducts.
 - Nonspecific axillary LN.

Opinion:

- Right breast global asymmetry with overlying suspicious microcalcifications likely of malignant etiology (BIRADS 4c) for histopathological examination .
- Normal sonomammography of the left breast (BIRADS 1)

Dr. Mohamed Mohsen, M.D.



MDT FORM

9/ 7/ 2020

Name	محمد محمد محمد	MRN	1000 / 1000
Age	41 yrs	ID	0000000000
Menopausal status	Post menopause	Phone	01001000000
BRCA SCORE		Marital status	Divorce
BMI		Family history	-ve
CO morbidity	NAD	REASON OF MDT	new

Clinical Data

EX: IDC G II DCIS / Focal

ER:	PR:	HER 2:	Ki67:
8	8	+	15%

Rt breast ACR B
BIRADS 4c

(Suspicious pleomorphic microcalc. patterns)
Met's Workup: B.I Preliminary nodules
for follow up



Recommendations:

as c.p, skin chd, skin thick, n.d. rel
b7c-

F- NA AC X3 TTX X3 No ARM

MDT Outcome & Plan

1st - WACR AC X3 TTX X3 No ARM
2nd - WACR AC X3 TTX X3 No ARM

MEDICAL ONCOLOGY		SURGERY		RADIOLOGY		PATHOLOGY		Nutrition		Psychiatric	
C. Co / 7 / 20		Dr. Hossam		Dr. Hossam		Dr. Hossam		Dr. Hossam		Dr. Hossam	

Patient Name : الأستاذ / روجيه حسن طه حسن
Patient No. : 28403071401947
Birth No. : 1161/25
Sex : Female
Age : 41 Y
Referring Doc :

Request No. : 13002724
Request Date : 26/03/2025
Verification Date : 22/04/2025
Financial Category : اورام دار السلام - مبادره
Department : Out Patient
Registered by : marwa mohamed youss

Pathology Report

Clinical Data
Right breast mass

Nature of specimen
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IHC technique: Sections prepared from paraffin block for immune-staining using Bench Mark ULTRA system for ER, PR, HER2, ER clone (1E2), PR clone (SP1), HER-2 clone (4B5), & Ki67 clone (30-9).

Microscopic examination
* Histological type: Invasive duct carcinoma
* Grade (differentiation + nuclear pleomorphism + mitosis): (3+2+2) Grade II
* DCIS: Focal
* Angioinvasion: Couldn't be detected in the received biopsy
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* IPT (Positive control showed appropriate reactivity) :
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* Ki 67: 15 % nuclear reaction of tumor cells

Diagnosis

Verified by :
Dr. Heba Hasan

مراجعة فتح مطلق

د. انوار

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غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient ID: 86007	Patient Name English: Rawhiah Hassan Taha Hassan
Age: ٤١ سنة, ٢ شهر	Report Date : 2025/07/09 09:08:51 AM

Contrast enhanced Spectral Mammography

CLINICAL SUMMARY: A 42 year-old lady with pathologically proven left breast IDC, Grade II.

CC & MLO low energy & recombined (post contrast) images

FINDINGS:

Background Uptake: mild background enhancement

ACR: b (scattered fibro-glandular densities)

RIGHT BREAST:

- **Low energy (Non contrast) images show:** Diffuse global asymmetry with architectural distortion, overlying pleomorphic microcalcifications along with breast diffuse edema pattern in the form of thickened skin and coarsened trabeculae; all mentioned findings seen involving the whole breast.
- **Recombined (Post contrast) images:** show corresponding diffuse heterogeneous non mass enhancement involving the whole breast parenchyma and reaching the nipple with consequent retraction. A heterogeneously enhancing axillary mass is also noted measuring 3.5 x 2.5 cm.
- **Complementary US:** show corresponding
 - Altered whole breast parenchyma associated with diffuse skin thickening reaching 5 mm and coarsen trabeculations.
 - Axillary irregular hypoechoic mass lesion ? primary malignant lesion ? nodal with extracapsular invasion.
 - Three level I pathologically enlarged axillary nodes are noted; two among them showed focal cortical thickness reaching 8 mm and the third one was globular showing lost hilum measuring 9 x 8 mm. A single non-specific level II node was noted measuring 9 x 5 mm with cortical thickness of 1.5 mm.
 - No pathological supra or infraclavicular lymph nodes could be detected.

LEFT BREAST:

- **Low energy (Non contrast) images** No masses, asymmetry or distortion or suspicious microcalcifications.
- **Recombined (Post contrast) images** show, no pathological uptake.
- Normal skin / nipple / areolar complex.

IMPRESSION:

RIGHT BREAST: (BIRADS 6) Locally advanced diffuse malignant process associated with edematous breast changes, nipple retraction and ipsilateral few pathological level I axillary lymph nodes.

LEFT BREAST: (BIRADS 1) No pathological mass or non-mass enhancement..

Best regards,

Dr. Amal Hegazi, M.Sc.

Dr. Sherihan Fakhry, MD.

MDT FORM

28/1/2026 G

Name	روحية حسن ب	MRN	1250/000
Age	41	ID	CA 6.4.4 (2.19)
Menopausal status		Phone	01114517830
NRS SCORE		Marital status	
BMI		Family history	-ve CH
CO morbidity	m. free	REASON OF MDT	Post op

Clinical Data

BX: S/P Rt. MRM (Post)

ER: PR: HER 2: Ki67:

Sonomammography: ± DC & II
Low grade DCIS

Mets Workup:
- nipple & axilla: free
- Deep margin: free
- Axillary LN: (8/16) with extension nodal extension

Recommendations:

- Yp T3 N1 M0

Rt cT₄ N₁ M₀ 8/18 PR 8/18 He2-u

→ Yp T3 N8/16 CC

MDT OUTCOME & PLAN

① Port to Rt CW, PCLN & axilla

② Tamoxifen / AI (femara) for 5 yrs. (we may consider ext. adjuv. after stage 2)

③ Vertebro for 2 yrs of N/A

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric
28/1/2026	28/1/2026				

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Patient No. : 28403071401947	Request Date : 26/03/2025
Path No. : 1161/25	Verification Date : 22/04/2025
Sex : Female	Financial Category : اورام دار السلام - مبادره
Age : 41 Y	Department : Out Patient
Referring Doc :	Registered by : marwa mohamed youss

Pathology Report

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- * Ki 67: 15 % nuclear reaction of tumor cells

Diagnosis

Verified by :
Dr. Heba Hasan

برحمة فتح ملف

د. انوار

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Patient No. : 28403071401947	Request Date : 26/03/2025
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Sex : Female	Financial Category : اورام دار السلام - مبالغه
Age : 41 Y	Department : Out Patient
Referring Doc :	Registered by : marwa mohamed youssef

Pathology Report

Right breast mass, Tru cut biopsy:
Invasive duct carcinoma, Grade II
IPT:

- * ER: Positive (score 8/8).
- * PR: Positive (score 8/8).
- * HER2: (score 1+).
- * Ki67: Positive in 15% of tumor cells.

Signature
Dr. Walaa Ghoneim
Dr. Heba Hassan

Verified by :
Dr. Heba Hasan

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